

3. *Narrow framing*, a subset of framing bias, can cause even long-term investors to obsess over short-term price fluctuations in a single industry or stock. This behavior works in concert with myopic loss aversion (see Chapter 17): The risk here is that by focusing only on short-term market fluctuations, excessive trading may be the result. This trading behavior has proven to be less than optimal for investors.
4. Framing and loss aversion can work together to explain excessive risk aversion. An investor who has incurred a net loss becomes likelier to select a riskier investment, whereas a net gainer feels predisposed toward less risky alternatives.

As previously noted, people answer questions differently based on how the questions are framed—either positively or negatively. Notice that the preceding dilemma is positively framed, focusing on the expected number of “lives saved” in each scenario. When the question was framed in this manner, 72 percent of physicians chose “A,” the safe-and-sure strategy; only 28 percent preferred to accept the risks inherent in option “B.”

Another statistically identical population of physicians was posed the same basic question. In this trial, however, the scenario was framed negatively:

*Imagine that the U.S. is preparing for the outbreak of an unusual Asian disease, which is expected to kill 600 people. Two alternative programs to combat the disease have been proposed. Assume that the exact scientific estimates of the consequences of the programs are as follows: If program C is adopted, 400 people will die. If program D is adopted, there is a one-third probability that nobody will die and a two-thirds probability that 600 people will die. Which of the two programs would you favor?*

As you can see, the two scenarios examine an identical dilemma. Saving 200 of 600 infected people, after all, implies that the remaining 400 of 600 are lost. However, when the question was framed negatively and physicians were concentrating on losses rather than gains, they voted in a dramatically different fashion. In the second trial, 22 percent of the physicians elected the conservative strategy (“400 will die”), and 72 percent felt more comfortable with the more volatile set of possible outcomes. This is almost a perfect, symmetric reversal of the breakdown that prevailed with positive framing.